

National Clinical Guideline — Dyslipidaemia and Lipid Management

National Health Authority · Clinical Guidance (synthetic)

§1 Cardiovascular risk stratification

Patients are stratified into low, moderate, high and very-high cardiovascular risk. Patients with established atherosclerotic cardiovascular disease, or diabetes with target-organ damage, are automatically very-high risk. A 10-year ASCVD risk at or above 20 percent denotes high risk, and 7.5 to 20 percent denotes moderate risk.

§2 LDL-C as the primary target

Low-density lipoprotein cholesterol is the primary target of lipid-lowering therapy. Treatment intensity is determined by baseline risk, with progressively lower targets for higher-risk patients.

§3 LDL-C targets by risk band

Very-high-risk patients: LDL-C below 1.4 mmol/L and at least a 50 percent reduction from baseline.

High-risk patients: LDL-C below 1.8 mmol/L. Moderate-risk patients: LDL-C below 2.6 mmol/L.

Low-risk patients: LDL-C below 3.0 mmol/L.

Any patient in a high or very-high risk band who is not receiving a statin represents a secondary-prevention care gap and should be prioritised for initiation.

§4 Statin therapy and intensification

Begin a high-intensity statin such as atorvastatin 40-80 mg or rosuvastatin 20-40 mg daily in high and very-high-risk patients. If the LDL-C target is not met after 4 to 6 weeks, add ezetimibe. Where the target is still not achieved in very-high-risk patients, add a PCSK9 inhibitor. Check a lipid panel and liver enzymes at baseline and after each change.

§5 Statins and special populations

Statins are generally safe to continue during acute illness and acute kidney injury. In patients with diabetes, the cardiovascular benefit of statins far outweighs the small increase in measured glucose.

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